



POLICY

Dysphagia Management for Inpatient Mental Health Services

Scope (Staff):	All NMHS Mental Health (MH) Clinical Staff
Scope (Area):	All NMHS Mental Health (MH) Inpatient Areas

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1. Aim

The aim of this policy is to ensure timely and accurate identification, management and referral of patients in mental health services at risk of dysphagia.

This policy establishes minimum practice standards for performing dysphagia screening and assessment by undertaking the following:

- Identification of patients with swallowing difficulties, or who are at risk of swallowing difficulties and/or aspiration.
- Early identification and referral of patients with dysphagia is essential to promote optimal care.

2. Background

Dysphagia may result from a wide range of medical conditions including acute or progressive neurological conditions, trauma, disease or surgery and is evident by recognisable symptoms of dysphagia (**Appendix 1: Dysphagia Symptomology Checklist**).

Dysphagia may cause any of the following which may lead to increased morbidity and mortality:

- Dehydration
- Malnutrition
- Choking
- Aspiration pneumonia.

Dysphagia screening and assessment of swallow function is essential for the accurate identification and diagnosis of dysphagia.

Dysphagia Screening must be undertaken by nurses deemed competent in the use of the Dysphagia Screening Tool (DST) endorsed for use within the service (**Appendix 2: Dysphagia Screening Tool**).

Daily Swallow Screen (DSS) is endorsed for use within the service and must be undertaken by nurses deemed competent in the use of the DSS (**Appendix 3: Dysphagia Swallow Screen**).

Dysphagia Assessment and management is a comprehensive assessment process undertaken by a Speech Pathologist, skilled in evaluating swallowing function, identifying factors contributing to dysphagia and developing individualised dysphagia management plans in collaboration with the patient, family, carer and treating team.

3. Risk

Failure to adhere to this policy may contribute to adverse outcomes for the patient which can lead to increased morbidity and mortality.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐

Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☒	Other	☐

4. Definitions

Aspiration pneumonia: is an infection of the lungs that can occur when the swallowing process fails and a substance (fluid, food, medicines, saliva, vomit or small foreign object) is inhaled into the lungs. It is a serious health condition which can be fatal.

Clinical handover: refers to any situation in which professional responsibility and accountability for some or all aspects of care for a patient, or group of patients, is transferred to another person or professional group on a temporary or permanent basis. The purpose of clinical handover is to achieve effective, high-quality communication of relevant clinical information that is understood and accepted by the receiver when responsibility for patient care is transferred in accordance with the mandatory DoH policy [WA Health Clinical Handover Policy](#) under the WA [DoH Clinical Governance, Safety and Quality Policy Framework](#).

Daily Swallow Screen: A Daily Swallow Screen (DSS) is endorsed by the service and is undertaken by nurses (deemed competent in the use of the DSS) during meals for patients on modified diets

Dysphagia: Dysphagia is the term used to describe difficulty swallowing food, fluid or saliva at the oral, pharyngeal and/or oesophageal stage.

Dysphagia Assessment: is a comprehensive process undertaken by a Speech Pathologist to identify specific difficulties with swallowing (drinking, chewing, eating, controlling saliva, taking medication, protecting the lungs from food or drink).

Dysphagia Screening: is a process to identify patients experiencing difficulties in swallowing. Dysphagia screening may be undertaken by a Speech Pathologist or a senior Registered Nurse competent in the use of the endorsed Dysphagia Screening Tool (DST).

Dysphagia Screening Tool: A Dysphagia Screening Tool (DST) is endorsed by the service to guide clinical staff (deemed competent in the use of the DST) in the identification and management of patients at risk of dysphagia.

iSOBAR: iSOBAR refers to the 6 key steps in the clinical handover process that specify minimum requirements for safe and effective clinical handover; Identify; Situation; Observations; Agreed Plan / Assessment and Readback/Risk/Recommendation (adapted from the WA Health Clinical Handover Policy).

Swallowing: (deglutition) is the process that allows for a substance (fluid, food, medicines, saliva) to pass from the oral cavity to the pharynx, and into the esophagus, while shutting the epiglottis to assist the movement of fluid via the oesophagus to the stomach.

Swallow Reflex: The automatic temporary closing of the epiglottis is controlled by the swallowing reflex and when this function is impaired the epiglottis remains open and food, fluid or other foreign material is inhaled, increasing the risk of an infection of the lungs (i.e., Aspiration pneumonia).

Swallow Screen: is undertaken as a component of Dysphagia Screening and assists in identifying fluids and foods that are appropriate for the individual pending completion of a



comprehensive Dysphagia Assessment.

5. Principles

All patients with dysphagia or suspected of having dysphagia **must** be screened on admission and referred to a speech pathologist for assessment.

Dysphagia Screening must be undertaken by nurses competent in the use of the Dysphagia Screening Tool (DST) endorsed for use within the service (**Appendix 2: Dysphagia Screening Tool**).

Daily Swallow Screen must be undertaken by nurses competent in the use of the Daily Swallow Screen (DSS) endorsed for use within the service (**Appendix 3: Dysphagia Swallow Screen**).

Patients assessed as having dysphagia must have a risk alert entered in the Psychiatric Services Online Information System (PSOLIS) in accordance with the [MH Management of Clinical Alerts Policy](#) (**Appendix 4: Dysphagia Risk Alert Sticker**).

Patients assessed as having dysphagia must have a Clinical Alert Form completed and placed at the front of the medical record (**Appendix 5: Clinical Alert Form PMR01**).

Special dietary requirements (including recommended diet and fluid texture modifications) must be recorded on the iSOFT Clinical Manager (iCM) and must be included in Nursing handover 'diet/diet notes' (Refer to [DoH Clinical Handover Policy](#) and the [DoH Clinical Handover Guideline](#)).

Special dietary requirements are documented in the management plan and must be ordered in accordance with the endorsed referral process for the service.

Communication regarding the risk of dysphagia is essential with mandatory verbal and written communication required to effectively identify and respond to patients at risk of choking and aspiration in accordance with the [MH Management of Clinical Alert Policy](#) and the [NMHS Clinical Documentation Policy](#).

Clinical teams will use the standardised clinical handover framework (iSOBAR) as part of a comprehensive, system-wide strategy, which will aid effective, concise and inclusive clinical communication in all clinical situations and contribute to improved patient safety. In and of itself, effective handover does not guarantee quality care as the responsibility and information handed over **must** be acted upon as directed by the [DoH Clinical Handover Policy](#) and the [DoH Clinical Handover Guideline](#) in accordance with the [WA DoH Clinical Governance, Safety and Quality Policy Framework](#).

6. Dysphagia Screening, Assessment and Management

6.1 Inpatient

Patients at risk of dysphagia are defined as showing signs and symptoms of dysphagia as identified in the Dysphagia Symptomology list (**Appendix 1**). Patients at risk of dysphagia must be screened on admission using the endorsed Dysphagia Screening Tool (DST). Dysphagia Screening must be undertaken by nurses deemed competent in the use of the Dysphagia Screening Tool (DST) endorsed for use within the service (**Appendix 2**).



Patients transferred from other health services/hospitals with established dysphagia assessed by a speech pathologist from that service and with established dysphagia management recommendations in place are checked to ensure that dietary requirements identified are current and that there are no reported difficulties in continuing with established dysphagia management recommendations.

Patients meeting the Indicators of Criteria / Aspiration Risk on the DST at-risk criteria **must** be referred to Speech Pathology for assessment using the endorsed referral process for the service.

6.2 Dysphagia Screening - Nursing

Patients at risk of dysphagia must be screened on admission by a nurse deemed competent in the use of the endorsed DST (**Appendix 2**).

For patients at risk of dysphagia as identified in the Dysphagia Symptomology Checklist (**Appendix 1**) the nurse must implement an interim dysphagia management plan in consultation with the treating Medical Officer and record the identified dysphasia risk in PSOLIS, in the medical record and on the patient journey board for clinical handover communication in accordance with the [MH Management of Clinical Alerts Policy](#) and the [DOH Clinical Handover Policy](#).

For patients identified as being at risk of dysphagia on modified diets the Dysphagia Swallow Screen (DSS) must be undertaken during all meals by a nurse deemed competent in the use of the endorsed DSS (**Appendix 3**).

Risk of Dysphagia must be included in the nursing handover using iSOBAR framework and recorded in iSOFT (diet/diet notes') in accordance with the requirements of the [NMHS Clinical Documentation Policy](#), the [DOH Clinical Handover Policy](#) and the [WA Clinical Handover Guideline](#).

Patients identified as at risk of Dysphagia **must** be referred to Speech Pathology for assessment using the endorsed referral process for the service.

The nurse advises Catering Services of the interim texture modified diet and fluid requirements via the Automated Menu System (AMS) and contacts Dietetics using the endorsed referral process for the service.

The nurse documents the interim management plan in the medical record and attaches the Dysphagia Risk Alert Sticker (**Appendix 4**) in accordance with the [MH Management of Clinical Alerts Policy](#).

The nurse completes the Clinical Alert Form PMR01 (**Appendix 5**) in accordance with the requirements of the [MH Management of Clinical Alerts Policy](#).

Dysphagia assessment is completed by a Speech Pathologist that includes a clinical examination of a patient's presenting behaviour, function and cognition as it relates to swallowing.

The Speech Pathologist will collaborate with the patient, family, carers and the treating team in the development and documentation of a Care Plan and Report (**Appendix 6: Speech Pathology Care Plan and Report PMR14**).



The Speech Pathologist will collaborate with Dietetics and Catering services regarding modified dietary requirements and confirm with nursing staff that the Automated Menu System (AMS) is updated in accordance with the care plan and consistent with International Dysphagia Diet Standardisation Initiative used by Catering services in NMHS (Refer to **Appendix 7: IDDSI International Framework**).

7. Documentation

The dysphagia risk alert status **MUST** be recorded in the following Mental Health documents in accordance with the requirements of the [MH Management of Clinical Alerts Policy](#).

- Dysphagia Risk Alert Sticker in the patient medical record (**Appendix 4**)
- Clinical Alert Form -PMR01 (**Appendix 5**). The Clinical Alert form **must** be placed at the front of the current medical record behind the risk divider
- iSOFT nursing handover using iSOFT Clinical Manager (iCM)
- SSCD Risk Assessment and Management Plan (RAMP SMHMR905)
- SSCD Treatment Support and Discharge Plan (SMHMR907)
- SSCD Care Transfer Summary (SMHMR916)
- Discharge summaries (NACS)

The PSOLIS “Alert” notification provides an additional means of communicating patients at risk of dysphagia that can be accessed by mental health clinicians across mental health services.

7.1 Discharge/Transfer Processes

In preparation for discharge or transfer to other services patient, family and carers must be informed and the management plan and ongoing dietary requirements communicated.

In the event of discharge or transfer to another health/hospital setting or residential care facility, group home or to NDIS providers, clinical handover on should be provided to that service on current dysphagia risks, dysphagia management recommendations and other relevant information.

Aboriginal Liaison officers involved in the transfer or discharge arrangements for Aboriginal Patients must be informed of the management plan and ongoing dietary requirements to ensure that patient dietary requirements are communicated to the receiving service.

8. Professional Education and Training

Profession Education and Training (PEaT) coordinates the following activities:

Identified Registered Nurses must complete the MH e-learning module for Dysphagia Screening Tool (DST) and competency training in the use of the screening tool (PEaT training module).

Nursing staff complete eLearning module and training relating to dysphagia and the use of DSS

Nursing staff attend annual training/refresher in accordance with [MH Recognising and Responding to Acute Deterioration Policy](#) that includes Basic Life Support, airway management, early recognition and management of Sepsis (Refer to the Sepsis Pathway in the [MH](#)



[Recognising and Responding to Acute Deterioration Policy](#) and [MHPHDS Mandatory Training Policy](#)).

9. Compliance and Evaluation

- 100% of patients with indicators of dysphagia / aspiration risk are screened using the DST on admission (documentation audit).
- 100% of dysphagia risk alerts are entered in the patient medical record and into PSOLIS within 24 hours of identification of risk (documentation audit).
- 100% of Registered Nurses undertaking the DST are deemed competent
- 100% of nurses (RN/EN) undertaking the DSS have completed training
- MHPHDS Safety, Quality and Performance report on audit and performance data to the Mental Health Clinical Governance Committee and MHPHDS Executive ([SQP Audit tools](#)).

Related internal policies, procedures and guidelines

[MH Dysphagia Screening Procedure](#)
[MHPHDS Mandatory Training Policy](#)
[MH Management of Clinical Alerts Policy](#)
[NMHS Clinical Documentation Policy](#)
[MH Recognising and Responding to Acute Deterioration Policy](#)
[MH Clinical Risk Assessment and Management Policy](#)
[MH Physical Healthcare Management Policy](#)
[MH Role of the Allocated Nurse in Inpatient Services Procedure](#)
[MH Person of Concern Protocol](#)
[MHPHDS Hand Hygiene Policy](#)
[NMHS Clinical Documentation Policy](#) (contains links to WA Clinical Handover Framework)
[MHPHDS Interpreter Services and Translated Information Policy](#)
[DoH State-wide Standardised Clinical Documentation \(SSCD\) Triage to Discharge Framework](#)

References

[DOH Clinical Handover Policy](#)
[WA Clinical Handover Guideline](#)
[The DoH Consent to Treatment Policy](#)
[ACSQHC Standard 8 Recognising and Responding to Acute Deterioration](#)
[Royal Brisbane Women's Hospital \(RBWH\) Dysphagia Screening Tool 2006](#)

Useful resources

International Dysphagia Diet Standardisation Initiative (IDDSI) www.iddsi.org
MH Dysphagia Screening Tool – PMR14A
MH Dysphagia Swallow Screen Chart – PMR14B

Policy Sponsor	Director of Nursing MHPHDS				
Policy Contact	Director of Nursing MHPHDS				
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Approved by:	Director of Nursing			Date:	28/07/2022
Endorsed by:	Director of Clinical Services			Date:	29/07/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☒ Std 5: Promotion and Prevention ☒ Std 6: Consumers ☒ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☒ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
29/07/2022	1.0	Director of Nursing MHPHDS	Initial endorsement, policy identifies mandatory requirements for the management of Dysphagia in Inpatient Mental Health Services by MDT
13/02/2023	1.1	A/Director of Nursing MHPHDS	Amendment to include reference to community assessment and collaborative care planning.
09/03/2023	1.2	Policy and Audit Officer MHPHDS	Endorsed Dysphagia Screening Tool PMR14A and Dysphagia Swallow Screen Forms PMR14B added
08/05/2023	1.3	Director of Nursing MHPHDS	Removal of reference to Community MHS assessment and collaborative care planning.

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative ISD 1545.

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Appendix 1: Dysphagia Symptomology Checklist

Evidence on Food and Fluid Intake

General:

- Difficulties managing oral secretions or drooling
- Absence or weakness of a voluntary cough or swallow
- Changes in voice quality/ tone (hoarseness/ moist sounding)
- Decreased mouth and tongue movements
- Tongue thrust/ primitive oral reflexes
- Frequent throat clearing
- Poor oral hygiene
- Raised temperature
- Frequent chest infections

When Eating or Drinking:

- Slow to initiate a swallow and / or delay in swallow (over 5 seconds)
- Uncoordinated chewing or swallowing
- Multiple swallows for each mouthful
- Pocketing of food in cheeks or throughout mouth
- Oral or nasal regurgitation of food / fluids
- Extended time to eat / drink
- Coughing or sneezing during / following eating

Following the consumption of food or drink:

- A wet hoarse sounding voice
- Fatigue
- Changes in respiratory pattern
- Regurgitating intake

Additional indicators of dysphagia:

Positioning/ Postural Factors

- Inability to achieve optimal positioning for safe swallowing (sitting fully upright, head in midline with chin tuck)
- Inability to sustain optimal positioning for safe swallowing (due to motor, cognitive or behavioural problems)
- Inability to remain seated fully upright for at least 30 minutes after food and fluid intake.

Alertness & Behavioural Factors

- Sedated / drowsy
- Unmotivated for oral intake / food refusal (check which consistencies are refused and why)
- Significantly agitated at mealtimes (i.e., vocalising / calling out during food or fluid intake)
- Over distractibility
- Poor response inhibition associated with compromised airway protection (talking or vocalising during food / fluid intake)
- Impaired orientation and memory ('forgetting' to swallow bolus)
- Poor impulse control and response inhibition (i.e., grabbing inappropriate food choice, or overly large boluses for swallowing capacity)
- Rule breaking behaviour (inappropriate for selection for swallowing function)
- Impaired new learning / ability to adopt compensatory strategies or adhere to staff prompts (i.e., rate reduction, over chewing bolus, supraglottal swallow technique)
- Impaired judgement of how much and how fast to consume food / fluid (tachyphagia, shovelling, overloading mouth / pharynx)
- Impaired judgement of appropriate food / fluid selection to achieve safe swallowing status

Oral preparatory / Oral transit / Pharyngeal / Oesophageal phase factors

- Difficulty managing own secretions : drooling
- Spillage of food / fluid from lips while chewing / swallowing
- Spillage of food / fluid posteriorly over base of tongue while chewing (evidenced by gagging or coughing before the swallow is initiated)
- Inefficient chewing / oral preparation of the bolus (due to (a) cognitive / behavioural factors / poor judgement (b) impaired oral motor control and / or (c) poor dentition)
- Effortful initiation of swallow
- Report of pain on swallowing
- Evidence of food / fluids residue in the mouth after swallowing
- Audible "clunking" swallow
- Coughing / choking during / after food / fluid intake
- Report of solids being 'stuck in throat'
- Spontaneous use of fluid to aid pharyngeal transit ('wash food down')
- Coughing on reclining / lying supine / bending over / changing posture (suggests pharyngeal pooling)
- Report of having to 'wash down' food
- Excessive burping / eructation
- Report of 'heartburn' or reflux
- Apparent deterioration in swallowing skills.

Upper Limb Motor Control Factors

- Difficulties manipulating unmodified utensils
- Inability to independently cut food into bite size pieces (takes overly large mouthfuls)
- Poorly controlled plate / cup to mouth transfer (haphazard / uncontrolled presentation of food / fluids to mouth)

Appendix 2: PMR14A Dysphagia Screening Tool (DST) page 1



**NORTH METROPOLITAN HEALTH SERVICE
MENTAL HEALTH**

..... Hospital

**MH DYSPHAGIA
SCREENING TOOL (DST)**

Only to be used by trained clinical staff

For Patients with Dysphagia and/or Aspiration Risk

Surname	Sex	U.R. No
Forenames	D.O.B.	
Address		
Ward	Registrar	Consultant
Use Patient I.D. Label when available		

Phase 1 Screening Procedure Does patient have any of the following or previous history of the following? (tick if applicable)

Dysphagia or aspiration risk on previous admission(s)	☐ Yes ☐ No
COPD +/- upper GI disorder	☐ Yes ☐ No
Stroke	☐ Yes ☐ No
Neurological involvement	☐ Yes ☐ No
Head injury	☐ Yes ☐ No
Head & neck cancer +/- surgery / chemotherapy / radiation	☐ Yes ☐ No
Orthopaedic conditions – # NOF, cervical spinal surgery	☐ Yes ☐ No
Acutely unwell, frail aged with co-morbidities	☐ Yes ☐ No
Suspected aspiration pneumonia / recurrent chest infections	☐ Yes ☐ No
Severe disability (e.g. physical disability)	☐ Yes ☐ No

– If 'Yes' to any of the above boxes proceed to Phase 2. If all boxes are ticked 'No', patient to have usual modified diet or normal diet & thin fluids. Complete 'Swallowing Management Plan' below. Dysphagia Swallow Screen (DSS) Initiated

Phase 2 Interview with Patient / Family / Carer (tick if applicable or assessable)

Do you have difficulty eating food / drinking fluid?	☐ Yes ☐ No
Do you cough or have choking episodes whilst eating / drinking?	☐ Yes ☐ No
Do you need to modify the food you eat? e.g. softer or smaller pieces	☐ Yes ☐ No
If 'Yes', provide details re current diet / fluid consistency: Diet: Fluid:	
Do you get chest infections?	☐ Yes ☐ No
If 'Yes', Is this recent? Yes ☐ No ☐	Has it been investigated before? ☐ Yes ☐ No

Indicators of Dysphagia / Aspiration Risk (tick if applicable)

Altered LOC / reduced responsiveness	☐ Yes ☐ No
Respiratory rate >30bpm	☐ Yes ☐ No
Slurred speech (dysarthria)	☐ Yes ☐ No
Weak / absent volitional cough	☐ Yes ☐ No
Not managing oral secretions (drooling, wet voice)	☐ Yes ☐ No
Weak voice (dysphonia)	☐ Yes ☐ No
Suspected aspiration pneumonia / recurrent chest infections	☐ Yes ☐ No
Documented or history of dysphagia / aspiration risk	☐ Yes ☐ No
Reports difficulty swallowing	☐ Yes ☐ No
Reported coughing / choking episodes whilst eating / drinking	☐ Yes ☐ No
Water Test: Administer only if 'no' has been ticked for all indicators of dysphagia. Give the patient 85mls (1/3 cup) water.	
Coughing during or between swallows, or up to one minute after swallowing	☐ Yes ☐ No
Wet / gurgly or hoarse voice after swallowing	☐ Yes ☐ No
Increased respiratory rate after swallowing	☐ Yes ☐ No

If 'Yes' to any of the above Indicators or the Water Test, medical officer must be advised. Patient to remain NBM. Refer to SP. If SP is NOT available, MO to determine hydration / nutrition / medication options e.g. NBM, IV fluids / NG feeds OR 900Mls Thickened Fluid and Smooth Puree Diet. If commenced on oral fluids / diet Dysphagia Swallow Screen to be undertaken at every meal.

If 'No' to all of the above Indicators and Water Test, place patient on usual modified diet or normal diet & thin fluid. DSS Initiated.

Swallowing Management Plan

Patient identified as dysphagic or aspiration risk. ☐ Yes ☐ No	Notify the Medical Officer. ☐ Yes ☐ No
If 'Yes': Patient placed NBM: ☐ Yes ☐ No	IV fluids: ☐ Yes ☐ No
900Mls Thickened Fluid and Smooth Puree Diet. ☐ Yes ☐ No	NG feeds: ☐ Yes ☐ No
Refer to Speech Pathologist ☐ Yes ☐ No	If Yes initiate DSS ☐ Yes ☐ No
Date referred:	
If 'No': ☐ Patient placed on general diet and thin fluids; ☐ OR usual modified: ☐ Diet	Fluids:

Print Name: Designation: Signature:

Ward: Unit: Date: Time:

Adapted from RBWH framework for use in MHS

PMR14A MH DYSPHAGIA SCREENING TOOL (DST)

G240
02/23

Appendix 2: PMR14A Dysphagia Screening Tool (Flow Chart) page 2

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH	<table border="1" style="width: 100%; border-collapse: collapse;"> <tr> <td style="width: 40%;">Surname</td> <td style="width: 10%;">Sex</td> <td style="width: 50%;">U.R. No</td> </tr> <tr> <td colspan="2">Forenames</td> <td>D.O.B.</td> </tr> <tr> <td colspan="3">Address</td> </tr> <tr> <td>Ward</td> <td>Registrar</td> <td>Consultant</td> </tr> <tr> <td colspan="3">Use Patient I.D. Label when available</td> </tr> </table>			Surname	Sex	U.R. No	Forenames		D.O.B.	Address			Ward	Registrar	Consultant	Use Patient I.D. Label when available		
Surname	Sex	U.R. No																
Forenames		D.O.B.																
Address																		
Ward	Registrar	Consultant																
Use Patient I.D. Label when available																		
..... Hospital																		
DYSPHAGIA FLOW CHART																		

Assessment and Follow-Up Plan

Phase 1:
Conditions commonly linked to dysphagia
Does the patient have any of the conditions linked to dysphagia outlined on screening tool?

YES

Phase 2: Patient / Carer Interview and Indicators of dysphagia. Does patient have any indicators of dysphagia outlined in Phase 2 of screen?

YES

Refer to Speech Pathologist. **NBN** until reviewed by Medical Officer to determine if NBN, IV Fluids, NG feeds, thickened fluids required.

NO

Proceed to Water Test.
Are there any indicators of dysphagia when the patient drinks 85mls of water?

YES

Swallowing Management Plan: Advise Medical Officer. The patient must remain **NBM** and referred to Speech Pathologist. If Speech Pathologist is not available, the Medical Officer to determine nutrition / hydration medication options.

Complete Swallow Management Plan.
Document in MR.
Include in Clinical Handover.

NO

Patient can commence usual diet. **Dysphagia Swallow Screen** initiated. Document in medical record.

Appendix 3: PMR14B Dysphagia Swallow Screen (DSS) page 1

[illegible]

Appendix 3: PMR14B Dysphagia Swallow Screen (DSS) page 2

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Hospital DYSPHAGIA SWALLOW SCREEN (DSS)		Surname		Sex	U.R. No
		Forenames		D.O.B.	
		Address			
		Ward	Registrar	Consultant	
		Use Patient I.D. Label when available			
INSTRUCTIONS FOR DYSPHAGIA SWALLOW SCREEN (DSS) STRICT NURSING OBSERVATION AT ALL MEALS Only to be used by nurses trained in the use of the Dysphagia Swallow Screen (PMR14b) Direct Observation of a patient drinking fluids and/or eating food <u>for every meal</u> . Record all YES responses on Dysphagia Swallow Screen e.g. If patient is unable to hold food in mouth record B1 on PMR14b					
IDENTIFIED RISKS					
No.	A – FLUIDS	Yes	No		
1	Unable to hold fluid in mouth				
2	Initiation of swallow is extremely slow				
3	Little observed laryngeal elevation				
4	Coughing / choking during or after drinking				
5	Unclear 'gurgly' voice quality after swallowing fluids				
No.	B – DIET	Yes	No		
1	Unable to hold food bolus in mouth				
2	Unable to chew food bolus				
3	Initiation of swallow is extremely slow				
4	Little observed laryngeal elevation				
5	Coughing / choking when eating				
6	Unclear 'gurgly' voice quality after eating				
7	Food remains in mouth after eating				
8	Patient complains of a sensation of obstruction whilst eating				
No.	C – BEHAVIOUR	Yes	No		
1	Very fast intake (tachyphagia)				
2	Overfilling mouth				
3	Eating non-food items (pica)				
4	Extreme distractibility e.g. constantly moving				

Appendix 4: Dysphagia Risk Alert Sticker

DYSPHAGIA ALERT

This patient has an identified
risk of choking.

Please see speech pathology report
before offering food or drink.

10. Appendix 5: Clinical Alert Form (PMR01) (Page 1)

[illegible]

Appendix 5: Clinical Alert Form (PMR01) (Page 2)

[illegible]

"To cancel / remove a Clinical Alert" cross with two diagonal lines, sign and date



Appendix: 6 Speech Pathology Care Plan and Report (Page 1)

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH		Surname		Sex	U.R. No
Hospital SPEECH PATHOLOGY REPORT/NOTES		Forenames		D.O.B.	
OLDER ADULT MENTAL HEALTH SPEECH PATHOLOGY SWALLOW INITIAL ASSESSMENT SUMMARY		Address			
		Ward	Registrar	Consultant	
		Use Patient I.D. Label when available			
MEDICAL DIAGNOSIS: _____					
ORAL MUSCLE FUNCTION:					
Jaw	Open	Tongue	Around lips	Reflex Cough	Effective
CN V	Close	CNXII	Around gums		Ineffective
	Lateral R		Into cheek R		Prompt
	Lateral L		Into cheek L		Delayed
Lips	Purse	Palate	Symmetry		Absent
CN VII	Retract	CN IX, X, XI	Closure	Cued Cough	Effective
Cheeks	Tone	Dentition			Ineffective
Comments: _____					
SWALLOW PHYSIOLOGY:					
	Impaired oral bolus preparation				
	Impaired oral bolus formation				
	Increased oral transit time				
	Reduced oral control				
	Reduced oral clearance				
	Reduced soft palate closure				
	Delayed swallow initiation				
	Reduced laryngeal excursion				
	Inco-ordinated laryngeal movement				
	Pharyngeal residue				
	Laryngeal penetration/aspiration				
	Reduced oesophageal motility				
	Reflux				
	Regurgitation				
COMMENTS:					
SAFETY FURTHER COMPROMISED BY:					
	Reduced and fluctuating alertness		Reduced co-operation		
	Fatigue's rapidly		Fluctuating behavioural state		
	Impaired cognitive state / confusion		Poor positioning		
	Reduced concentration / distractibility		Impulsiveness / inappropriate rate		
	Impaired airway protection		Reduced awareness		
	Shortness of breath		Overfilling mouth		
	Dystonia – cervical / orofacial / laryngeal		Anxiety / agitation		
	Inadequate dentition		Involuntary movements		
	Food pocketing/holding		Unable to use compensatory techniques		
SWALLOW DIAGNOSIS & SEVERITY: _____					
ADDITIONAL COMMENTS: _____					
SNR Speech Pathologist			Date of Assessment		

PMR 14 SPEECH PATHOLOGY REPORT/NOTES

Revised 2020

Appendix: 6 Speech Pathology Care Plan and Report (Page 2)

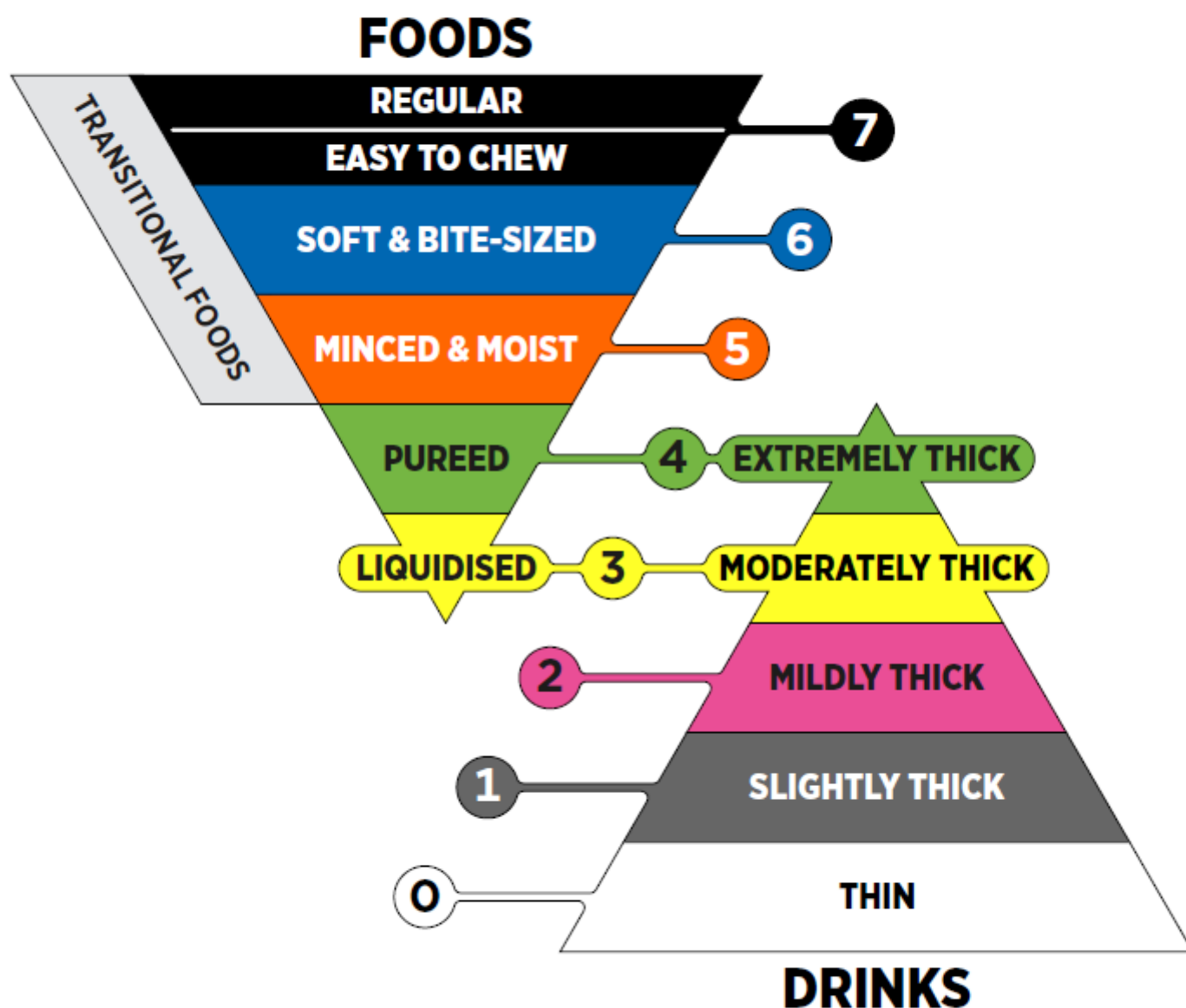
NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH _____ Hospital		Surname _____ Sex _____ U.R. No _____	
		Forenames _____ D.O.B. _____	
SPEECH PATHOLOGY REPORT/NOTES OLDER ADULT MENTAL HEALTH SWALLOWING CARE PLAN (*For Food and Fluid Level explanations, please refer to The International Dysphagia Diet Standardisation Initiative www.iddsi.org)		Address _____ Ward _____ Registrar _____ Consultant _____	
FLUIDS* ☐ THIN (Normal). Level 0 ☐ SLIGHTLY THICK. Level 1 ☐ MILDY THICK. Level 2 (Level 150) ☐ MODERATELY THICK. Level 3 (Level 400) ☐ EXTREMELY THICK. Level 4 (Level 900)		SAFE SWALLOWING STRATEGIES ☐ Sitting upright and alert with head in midline ☐ Orient patient to meal ☐ Reduce distractions ☐ Encourage chin down posture ☐ Encourage slow eating rate ☐ Encourage small mouthfuls – tspn vs dspn ☐ Ensure each mouthful is swallowed ☐ Double swallows ☐ Alternate diet and fluids ☐ Encourage small sips from a cup ☐ Straw for drinking ☐ Offer only top ½ glass ☐ Small plate and teaspoon ☐ Discourage talking while eating ☐ Regular swallow-cough-re-swallow ☐ Cease oral intake if: -drowsy -coughing -after 45 min - wet/gurgly voice quality - upper airway sounds - fatigued or - non-compliant ☐ Stay upright for approx 30 mins. after meals ☐ Complete mouth-care at end of all meals ☐ Medications in thickened fluid – LEVEL: ☐ Medications in yoghurt ☐ Crushed medications *Please consult the Pharmacist on the safety of combining the medication with the above, and on the safety of crushing medications before doing so.	
FOODS* ☐ LIQUIDISED. Level 3 ☐ PUREED. Level 4 ☐ MINCE & MOIST. Level 5 ☐ SOFT & BITE SIZED. Level 6 ☐ EASY CHEW- NO BREAD. Level 7 ☐ EASY CHEW + BREAD. Level 7 ☐ REGULAR (As desired). Level 7 ☐ Extra gravy / sauce			
SPECIAL EQUIPMENT ☐ Dysphagia cup ☐ Nose cut-out cup ☐ Plate guard ☐ Scoop bowl ☐ Built up cutlery ☐ Non-slip mat ☐ Straw			
INDEPENDENCE ☐ Independent ☐ Stand-by supervision ☐ One to one supervision ☐ Partial assist ☐ Full assist			
REFERRAL TO ☐ Team for consideration of non-oral feeding ☐ Dietician for provision of nourishing supplements ☐ Dental clinic for denture review ☐ Videofluoroscopy clinic		SEE ADDITIONAL STRATEGIES (see details on ward Care plan) ☐ Specific Positional strategies ☐ Behavioural Strategies ☐ Medication strategies ☐ Swallow exercises ☐ Water Protocol ☐ Other	
SNR Speech Pathologist _____		Date of Assessment _____	

PMR 14 SPEECH PATHOLOGY REPORT/NOTES

THIS CARE PLAN IS ACCURATE AT TIME OF ASSESSMENT, AND NO RESPONSIBILITY IS TAKEN FOR FLUCTUATING PATIENT STATUS. Revised 2020

The IDDSI Framework

Providing a common terminology for describing food textures and drink thicknesses to improve safety for individuals with swallowing difficulties.



© The International Dysphagia Diet Standardisation Initiative 2019 @ <https://iddsi.org/framework/>

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